

CRITICAL ALERT: INPATIENT EMERGENCY ADVANCED CLINICAL LAB & BIOMARKER REPORT

Patient Name:	Mrs. Sunita Rao	UHID / Reg No:	AP-993012-2026
Age / Gender:	62 Yrs / Female	Collected Date:	May 17, 2026 10:15 AM
Referred By:	Dr. Meera Reddy, DM	Ward Location:	ICU - Bed 04 (Emergency)

CLINICAL HISTORY & PRESENTATION

Patient was brought to the emergency department via cardiac ambulance presenting with acute substernal crushing chest pain radiating to the left arm, profound dyspnea, diaphoresis, and sudden oliguria. Known history of type-2 diabetes mellitus and chronic vascular hypertension. Clinical evaluation warrants immediate biochemical investigation due to high suspicion of acute decompensated coronary syndrome and concomitant critical organ failure.

CORE BIOCHEMISTRY & DIAGNOSTIC PANEL RESULTS

Test Parameter	Observed Value	Reference Interval	Units	Flag Status
Cardiac Troponin I (cTnI)	3.80	0.00 - 0.04	ng/mL	CRITICAL HIGH
NT-proBNP	4200	< 125	pg/mL	CRITICAL HIGH
Serum Creatinine	4.10	0.60 - 1.20	mg/dL	CRITICAL HIGH
Blood Urea Nitrogen (BUN)	68.0	7.0 - 20.0	mg/dL	HIGH
Fasting Plasma Glucose	285	70 - 100	mg/dL	CRITICAL HIGH
Total WBC Count	16400	4000 - 11000	/μL	ELEVATED
Hemoglobin	8.2	12.0 - 15.5	g/dL	CRITICAL LOW

CRITICAL CARE CLINICAL INTERPRETATIONS

- **Cardiovascular Panel:** Severe escalation of Cardiac Troponin I (3.80 ng/mL) accompanied by profoundly high NT-proBNP confirms acute myocardial injury consistent with an ongoing acute myocardial infarction (Heart Attack) and secondary acute congestive heart failure.
- **Renal Panel:** Serum Creatinine at 4.10 mg/dL with a calculated eGFR of 14 mL/min indicates an abrupt development of Stage 5 Acute-on-Chronic Kidney Injury (Renal Failure) requiring immediate nephrology intervention and urgent dialysis evaluation.
- **Metabolic State:** Uncontrolled hyper-glycemia (285 mg/dL) places the patient at immediate risk of diabetic ketoacidosis. Critical microcytic hypochromic anemia (Hemoglobin 8.2 g/dL) complicates tissue oxygenation demands.

PRIMARY CLINICAL DIRECTIVES

THIS IS A HIGH-RISK, LIFE-THREATENING CRITICAL EMERGENCY. Immediate medical attention is required. Transfer patient to Cardiac ICU post-haste for continuous hemodynamic monitoring, emergent coronary angiography evaluation, anti-thrombotic therapy protocol, and emergent nephrology consult.

Dr. V. Kapoor, MD, DM

Chief Consultant Neuro-Cardiology Care
Apollo Comprehensive Emergency Network